

SURGICAL PATHOLOGY REPORT

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TCGA-XF-AAN5-01A-PR

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DIAGNOSIS:

ANTERIOR PELVIC EXENTERATION WITH BILATERAL PELVIC LYMPH NODE DISSECTION AND URINARY DIVERSION:

RIGHT DISTAL URETER (AFS):

FROZEN SECTION DIAGNOSIS:

- NO HIGH GRADE ATYPIA OR TUMOR IDENTIFIED

FINAL DIAGNOSIS:

- BENIGN URETER EXHIBITING CHRONIC INFLAMMATION AND ASSOCIATED REACTIVE CHANGES
- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

LEFT DISTAL URETER (BFS):

FROZEN SECTION DIAGNOSIS:

- NO HIGH GRADE ATYPIA OR TUMOR IDENTIFIED

FINAL DIAGNOSIS:

- BENIGN URETER
- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

URETHRAL MARGIN (CFS):

FROZEN SECTION DIAGNOSIS:

- NO HIGH GRADE ATYPIA OR TUMOR IDENTIFIED

FINAL DIAGNOSIS:

- BENIGN URETHRAL MUCOSA WITH SQUAMOUS METAPLASIA, CHRONIC INFLAMMATION AND ASSOCIATED REACTIVE CHANGES
- NO DYSPLASIA OR MALIGNANCY IDENTIFIED

BLADDER, UTERUS, CERVIX, BILATERAL FALLOPIAN TUBES AND OVARIES (D):

BLADDER:

- INVASIVE POORLY DIFFERENTIATED UROTHELIAL CARCINOMA WITH FOCAL SQUAMOID FEATURES (6 X 3 X 2 CM), GRADES 3 & 4/4, EXTENDING GROSSLY AND MICROSCOPICALLY THROUGH RIGHT BLADDER WALL INTO PERIVESICAL SOFT TISSUE AND INVOLVING RIGHT URETEROVESICAL JUNCTION AND INTRAMURAL URETER WITH RESULTING OBSTRUCTION (INDWELLING RIGHT URETERAL STENT)
- SINGLE FOCUS OF VASCULAR INVASION IDENTIFIED
- ADJACENT MUCOSA EXHIBITING UROTHELIAL CARCINOMA IN SITU (FLAT LESION)
- RANDOM BLADDER MUCOSA INCLUDING TRIGONE AND BLADDER NECK SHOWING SQUAMOUS METAPLASIA WITHOUT EVIDENCE OF UROTHELIAL DYSPLASIA OR CARCINOMA IN SITU (FLAT LESION)
- RESECTION MARGINS, FREE OF UROTHELIAL DYSPLASIA AND MALIGNANCY

UTERUS WITH CERVIX:

- CERVIX, BENIGN SQUAMOUS AND ENDOCERVICAL MUCOSA
- ENDOMETRIUM, ATROPHY
- MYOMETRIUM, NO SIGNIFICANT HISTOPATHOLOGIC CHANGE
- SEROSA, NO SIGNIFICANT HISTOPATHOLOGIC CHANGE
- NO ATYPIA, DYSPLASIA OR MALIGNANCY IDENTIFIED

BILATERAL FALLOPIAN TUBES AND OVARIES:

- OVARIES, ATROPHY AND BENIGN PHYSIOLOGIC CHANGES
- FALLOPIAN TUBES, SIMPLE PARATUBAL CYST ON LEFT WITH NO OTHER SIGNIFICANT HISTOPATHOLOGIC CHANGE
- NO ATYPIA OR MALIGNANCY IDENTIFIED

LEFT PROXIMAL URETER (E):

- BENIGN URETER
- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

ICD-O-3
Carcinoma, urothelial NOS
8120/3
Site: Bladder NOS
C67.9
QW3/26/14

30% squamous, per TSS.

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RIGHT PROXIMAL URETER (F):

- BENIGN URETER EXHIBITING CHRONIC INFLAMMATION AND ASSOCIATED REACTIVE CHANGES
- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

LEFT EXTERNAL ILIAC LYMPH NODES (G):

- NO MALIGNANCY IDENTIFIED IN SIX LYMPH NODES EXAMINED (0/6)

RIGHT PRE-SCIATIC LYMPH NODES (H):

- NO MALIGNANCY IDENTIFIED IN ONE LYMPH NODE EXAMINED (0/1) [ENTIRELY SUBMITTED]

LEFT PRE-SCIATIC LYMPH NODES (I):

- NO MALIGNANCY IDENTIFIED IN ONE LYMPH NODE EXAMINED (0/1) [ENTIRELY SUBMITTED]

PRE-SACRAL LYMPH NODES (J):

- NO MALIGNANCY IDENTIFIED IN FOUR LYMPH NODES EXAMINED (0/4)

LEFT OBTURATOR HYPOGASTRIC LYMPH NODES (K):

- NO MALIGNANCY IDENTIFIED IN NINE LYMPH NODES EXAMINED (0/9)

LEFT PARA-AORTIC LYMPH NODES (L):

- NO MALIGNANCY IDENTIFIED IN SIX LYMPH NODES EXAMINED (0/6)

LEFT COMMON ILIAC LYMPH NODES (M):

- NO MALIGNANCY IDENTIFIED IN NINE LYMPH NODES EXAMINED (0/9)

HIGHEST INTER-AORTIC CAVAL LYMPH NODES, ORIGIN OF INFERIOR MESENTERY (N):

- NO MALIGNANCY IDENTIFIED IN ONE LYMPH NODE EXAMINED (0/1) [ENTIRELY SUBMITTED]

RIGHT PARA CAVAL LYMPH NODES (O):

- NO MALIGNANCY IDENTIFIED IN SIX LYMPH NODES EXAMINED (0/6)

RIGHT COMMON ILIAC LYMPH NODES (P):

- NO MALIGNANCY IDENTIFIED IN SEVEN LYMPH NODES EXAMINED (0/7)

RIGHT LYMPH NODE OF CLOQUET (Q):

- NO MALIGNANCY IDENTIFIED IN ONE LYMPH NODE EXAMINED (0/1)

RIGHT EXTERNAL LYMPH ILIAC LYMPH NODES (R):

- NO MALIGNANCY IDENTIFIED IN ONE LYMPH NODE EXAMINED (0/1) [ENTIRELY SUBMITTED]

RIGHT OBTURATOR/HYPOGASTRIC LYMPH NODES (S):

- NO MALIGNANCY IDENTIFIED IN 13 LYMPH NODES EXAMINED (0/13)

LEFT LYMPH NODE OF CLOQUET (T):

- NO MALIGNANCY IDENTIFIED IN ONE LYMPH NODE EXAMINED (0/1)

LYMPH NODE SUMMARY: NO MALIGNANCY IDENTIFIED IN TOTAL OF 66 LYMPH NODES EXAMINED (0/66)

PATHOLOGIC TNM STAGE: pTis,T3bN0MX

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COMMENT:

Representative sections of invasive carcinoma are submitted for P53 assay by immunohistology, results of which will be issued in an addendum.

SPECIMEN SOURCE:

A: "Right distal ureter F/S"
 B: "Left distal ureter F/S"
 C: "Urethral margin-f/s"
 D: "Bladder, uterus, cervix, bilateral fallopian tubes and ov"
 E: "Left proximal ureter"
 F: "Right proximal ureter"
 G: "Left external iliac lymph nodes"
 H: "Right pre-sciatic lymph nodes"
 I: "Left pre-sciatic lymph nodes"
 J: "Pre-sacral lymph nodes"
 K: "Left obturator hypogastric lymph nodes"
 L: "Left para-aortic lymph nodes"
 M: "Left common iliac lymph nodes"
 N: "Highest inter-aortic caval LN, origin of inferior mesente"
 O: "Right para caval LN"
 P: "Right common iliac LN"
 Q: "Right lymph node of cloquet"
 R: "Right external iliac lymph nodes"
 S: "Right obturator/hypogastric lymph nodes"
 T: "Left lymph nodes of cloquet"

CLINICAL INFORMATION:

Pre-op Dx: Bladder cancer

GROSS EXAMINATION:

A: The specimen is received fresh from the O.R. and labeled "Right distal ureter F/S". It consists of one segment of ureter measuring 0.4 cm in length x 0.5 cm in diameter. The specimen is entirely submitted for frozen section diagnosis in AFS.

B: The specimen is received fresh from the O.R. and labeled "Left distal ureter F/S". It consists of one segment of ureter measuring 0.5 cm in length x 0.3 cm in diameter. The specimen is entirely submitted for frozen section diagnosis in BFS.

C: The specimen is received fresh from the O.R. and labeled "Urethral margin-f/s". It consists of one piece of irregularly shaped tan tissue measuring 2 x 0.6 x 0.4 cm. The specimen is entirely submitted for frozen section diagnosis in CFS.

D: The specimen is received fresh from the O.R. and labeled "Bladder, uterus, cervix, bilateral fallopian tubes and ovaries". The entire specimen measures 21.5 x 13 x 5 cm. Peritoneum on the posterior surface measures 20 x 11 x less 0.1 cm and is smooth, glistening and mobile, being uninvolved by tumor. The bladder itself measures 8 x 6.5 x 4 cm. A palpable mass is present on the right side. This area of the outside of the bladder is inked black and the bladder is opened through the urethra and anterior bladder wall. The opened bladder shows an exophytic and ulcerated tumor 6 x 3 x 2 cm on the right lateral wall that involves the right ureterovesical junction. It comes within 1 cm of the urethral margin. Tumor extends through the bladder wall into the perivesical soft tissue to a depth of 2 cm. A ureteral stent protrudes from the right ureteral orifice. It is removed and the ureter is opened. The right attached ureter measures 7 cm in length x 1.5 cm in circumference in its more proximal aspect but narrows to less than 2-3 mm in its intramural segment where it is surrounding by tumor. The left UVJ is identified and is patent. It is opened to reveal the left attached ureter measuring 4.5 cm in length x 0.6 cm in circumference. The ureteral mucosa is

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unremarkable. The uninvolved bladder wall is 0.8 cm in thickness. No other lesions or masses are identified on the bladder mucosa. The uterus with cervix measures 5.5 x 3.5 x 2.5 cm. The external os of the cervix is 0.8 cm in diameter. The exocervix measures 2.5 cm in diameter. The mucosa is grossly normal. The uterus is pear shaped and symmetrical and the serosa is smooth. The uterus is then bivalved. The endocervical canal measures 2 cm in length x 0.3 cm in diameter. The endocervical mucosa is unremarkable. The uterine wall measures 1.1 cm in thickness. The endometrium is 1 mm in thickness. The myometrium is 1 cm in thickness. The endometrium, myometrium and serosal are normal. The right fallopian tube measures 4 cm in length x 0.4 to 0.5 cm in diameter. The right ovary measures 1.3 x 0.8 x 0.5 cm and is grossly unremarkable. The left fallopian tube measures 5 cm in length x 0.3 to 0.5 cm in diameter with a 1 mm simple paratubal cyst. The right ovary measures 1.2 x 0.6 x 0.5 cm and is grossly unremarkable. Representative sections are submitted in 18 cassettes.

E: The specimen is received in formalin and labeled "Left proximal ureter". It consists of one segment of ureter measuring 1 cm in length x 0.4 cm in diameter. The specimen is entirely submitted in one cassette.

F: The specimen is received in formalin and labeled "Right proximal ureter". It consists of one segment of ureter measuring 1 x 0.4 x 0.4 cm. The specimen is entirely submitted in one cassette.

G: The specimen is received in formalin and labeled "Left external iliac lymph nodes". It consists of multiple pieces of irregularly shaped fibroadipose tissue measuring in aggregate 2.5 x 2.5 x 1.5 cm. Multiple possible lymph nodes are identified measuring from 0.4 to 1 cm in dimension. The specimen is entirely submitted in three cassettes.

H: The specimen is received in formalin and labeled "Right pre-sciatic lymph nodes". It consists of multiple pieces of irregularly shaped fibroadipose tissue measuring in aggregate 1.5 x 1.5 x 1 cm. Multiple possible lymph nodes are identified measuring from 0.1 to 0.4 cm in dimension. The specimen is entirely submitted in one cassette.

I: The specimen is received in formalin and labeled "Left pre-sciatic lymph nodes". It consists of multiple pieces of irregularly shaped fibroadipose tissue measuring in aggregate 2 x 2 x 1 cm. Multiple possible lymph nodes are identified measuring from 0.1 to 0.5 cm in dimension. The specimen is entirely submitted in one cassette.

J: The specimen is received in formalin and labeled "Pre-sacral lymph nodes". It consists of multiple pieces of irregularly shaped fibroadipose tissue measuring in aggregate 3 x 3.5 x 1.5 cm. Multiple possible lymph nodes are identified measuring from 0.1 to 0.3 cm in dimension. The specimen is entirely submitted in six cassettes.

K: The specimen is received in formalin and labeled "Left obturator hypogastric lymph nodes". It consists of multiple pieces of irregularly shaped fibroadipose tissue measuring in aggregate 5 x 5 x 2 cm. Multiple possible lymph nodes are identified measuring from 0.1 to 3 cm in dimension. The specimen is entirely submitted in six cassettes.

L: The specimen is received in formalin and labeled "Left para-aortic lymph nodes". It consists of one piece of irregularly shaped fibroadipose tissue measuring 2.5 x 2 x 2 cm. One lymph node is identified measuring 2 x 2 x 2 cm. The specimen is entirely submitted in six cassettes.

M: The specimen is received in formalin and labeled "Left common iliac lymph nodes". It consists of multiple pieces of irregularly shaped fibroadipose tissue

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measuring in aggregate 4 x 3 x 2 cm. Multiple possible lymph nodes are identified measuring from 0.2 to 3 cm in dimension. The specimen is entirely submitted in seven cassettes.

N: The specimen is received in formalin and labeled "Highest inter-aortic caval LN, origin of inferior mesentery". It consists of one piece of irregularly shaped fibroadipose tissue measuring 1 x 0.5 x 0.4 cm. No obvious lymph nodes are identified. The specimen is entirely submitted in one cassette.

O: The specimen is received in formalin and labeled "Right para caval LN". It consists of one piece of irregularly shaped fibroadipose tissue measuring 4 x 4 x 1.5 cm. Multiple possible lymph nodes are identified measuring 0.2 to 2 cm in dimension. The specimen is entirely submitted in five cassettes.

P: The specimen is received in formalin and labeled "Right common iliac LN". It consists of multiple pieces of fibroadipose tissue measuring in aggregate 5 x 2 x 1 cm. Multiple possible lymph nodes are identified measuring from 0.5 to 5 cm in dimension. The specimen is entirely submitted in seven cassettes.

Q: The specimen is received in formalin and labeled "Right lymph node of cloquet". It consists of one piece of irregularly shaped fibroadipose tissue measuring 2 x 1.5 x 1 cm. One lymph node is identified measuring 2 x 1 x 0.5 cm. The specimen is entirely submitted in one cassette.

R: The specimen is received in formalin and labeled "Right external iliac lymph nodes". It consists of multiple pieces of irregularly shaped fibroadipose tissue measuring in aggregate 2 x 2 x 1 cm. Multiple possible lymph nodes are identified measuring from 0.2 to 1 cm in dimension. The specimen is entirely submitted in two cassettes.

S: The specimen is received in formalin and labeled "Right obturator/hypogastric lymph nodes". It consists of multiple pieces of irregularly shaped fibroadipose tissue measuring in aggregate 5 x 5 x 2.5 cm. Multiple possible lymph nodes are identified measuring from 0.2 to 1.8 cm in dimension. The specimen is entirely submitted in ten cassettes.

T: The specimen is received in formalin and labeled "Left lymph nodes of cloquet". It consists of one piece of irregularly shaped fibroadipose tissue measuring 2 x 1.5 x 1 cm. Multiple possible lymph nodes are identified measuring 0.4 to 1.5 cm in dimension. The specimen is entirely submitted in one cassette.

SECTIONS:

AFS: frozen section, right distal ureter
BFS: frozen section, left distal ureter
CFS: frozen section, urethral margin
D1: bladder, uterus, cervix, bilateral fallopian tubes and ovaries; right ureterovesical junction
D2: left ureterovesical junction
D3: trigone and bladder neck
D4: bladder neck
D5: anterior bladder wall
D6-9: right lateral bladder wall/lesion
D10: posterior bladder wall
D11: dome of bladder
D12: left lateral bladder wall
D13: anterior cervix
D14: posterior cervix
D15: anterior uterus
D16: posterior uterus
D17: right ovary and right fallopian tube

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D18: left ovary and left fallopian tube
 E: left proximal ureter
 F: right proximal ureter
 G1-3: left external iliac lymph nodes
 H: right pre-sciatic lymph nodes
 I: left pre-sciatic lymph nodes
 J1-6: pre-sacral lymph nodes
 K1-3: left obturator hypogastric lymph nodes; one lymph node each cassette
 K4-6: remainder of tissue
 L1-6: left para-aortic lymph nodes; sixth cassette containing one lymph node
 M1-7: left common iliac lymph nodes
 N: highest inter-aortic caval lymph nodes, origin of inferior mesentery
 O1-5: right para caval lymph nodes
 P1-7: right common iliac lymph nodes; P2-7 containing one lymph node each cassette
 Q: right lymph node of Cloquet; one lymph
 R1, 2: right external iliac lymph nodes
 S1-10: right obturator/hypogastric lymph nodes; S 5,6 containing one lymph node
 T: left lymph nodes of Cloquet

INTRAOPERATIVE FROZEN CONSULTATION:

AFS: Right distal ureter
 - no high-grade atypia or tumor identified

 BFS: Left distal ureter
 - no high-grade atypia or tumor identified

 CFS: Urethral margin
 - no high-grade atypia or tumor identified

MICROSCOPIC EXAMINATION:

AFS-CFS: See final microscopic-diagnosis.

D: Sections of the bladder taken from the right lateral wall show a poorly differentiated invasive urothelial carcinoma (D1,6-9) extending through the entire bladder wall into perivesical fat (D6,8,9). The tumor involves the right ureterovesical junction (D1) where it extends into the intramural portion of the right ureter and extends through the wall into periureteral and perivesical fat. The invasive tumor has an adjacent focus of urothelial carcinoma in situ (flat lesion) [D9]. While no lymphatic space invasion is seen, there is a focus of vascular invasion noted (D8). The peripheral inked perivesical margin is free of malignancy. Additional random sections of the bladder including the trigone and bladder neck show foci of squamous metaplasia but no urothelial dysplasia or urothelial carcinoma in situ. All final resection margins are free of both urothelial dysplasia and invasive carcinoma.

E-T: See final microscopic-diagnosis.

Criteria	Yes	No
Diagnosis Discrepancy		☒
Primary Tumor Site Discrepancy		☒
HIPAA Discrepancy		☒
Prior Malignancy History		☒
Dual/Synchronous Primary Noted		☒
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	Date Reviewed: 1/16/2014	